ACE Transitions

Support. Strength. Success.

PEOPLE BRIDGING BRIGHTER TOMORROWS

PEER SUPPORT SERVICES

POLICY & PROCEDURES MANUAL

H0038 / H0038 HQ Program Operations

DRAFT FOR BOARD REVIEW

Effective Date: _____ Board Approval Date: _____

Policy Owner: Executive Director / Program Manager Clinical Oversight: PSS Program Supervisor (QP)

This manual is designed for organizational implementation and payer-readiness. It must be read together with current NC Medicaid Clinical Coverage Policy 8G, health-plan contracts, NCTracks requirements, accreditation standards, and applicable federal and state law.

Document Control and Approval

Document Title	ACE Transitions Peer Support Services Policy & Procedures Manual
Version	1.0 - Draft for Board Review
Effective Date	To be established upon formal approval
Review Cycle	At least annually and whenever governing requirements materially change
Primary Authority	NC Medicaid Clinical Coverage Policy 8G, amended September 1, 2026
Applies To	Board, Executive Director, Program Manager, QP/PSS Program Supervisor, Peer Coordinator, CPSS staff, Intake, Claims, Administrative and Quality/Compliance personnel

Board Adoption

Board Chair / Authorized Officer: _____ Date: _____

Executive Director: _____ Date: _____

PSS Program Supervisor (QP): _____ Date: _____

Critical Medicaid Compliance Note

ACE Transitions shall not rely on a part-time QP as the sole PSS program director for Medicaid H0038 operations. Current NC Medicaid policy requires the PSS program to be under the direction of a full-time Qualified Professional. A part-time QP may provide supplemental coverage, but ACE must document a compliant full-time QP program-direction arrangement before Medicaid PSS is rendered or billed.

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PSS-001 Governance, Purpose and Regulatory Hierarchy

PURPOSE

Create a controlled framework for safe, recovery-oriented, Medicaid-compliant Peer Support Services (PSS).

POLICY

ACE Transitions shall operate PSS in accordance with the requirements in effect on the date of service. When this manual conflicts with law, NC Medicaid policy, a health-plan contract, accreditation standard, or a more restrictive payer requirement, the controlling external requirement governs. Leadership shall update this manual promptly when material requirements change.

Responsibilities

Role	Primary Responsibility
Board of Directors	Approves governing policies and receives compliance and quality reports.
Executive Director	Executive accountability, contracting, resource allocation and corrective-action authority.
Program Manager	Operational implementation, workflow coordination and policy administration.
QP/PSS Program Supervisor	Clinical/program direction required for PSS; supervision, plan oversight, quality review and beneficiary monitoring.
All Staff	Follow current requirements, report concerns, and refrain from unauthorized practice or billing.

Procedure

1. Executive leadership shall maintain a current regulatory library containing NC Medicaid Clinical Coverage Policy 8G, relevant Medicaid bulletins, NCTracks guidance, health-plan manuals, accreditation requirements, and applicable organizational agreements.
2. The Program Manager shall maintain a change log and route material regulatory updates to the Executive Director, QP/PSS Program Supervisor, Claims Specialist, Intake Specialist, and affected staff.
3. No operational practice may be adopted solely because it is convenient or customary if it conflicts with medical necessity, beneficiary choice, documentation, scope-of-practice, or billing requirements.
4. Policies shall be reviewed at least annually and sooner when NC Medicaid, a contracted health plan, accreditation body, or governing law materially changes program requirements.

Required Records: Policy register, regulatory update log, board approvals, payer manuals, accreditation files and corrective-action records.

PSS-002 Organizational Roles and Separation of Duties

PURPOSE

Define decision rights so administrative, clinical, peer-support and billing functions remain appropriately separated.

POLICY

ACE Transitions shall maintain written role descriptions and shall not permit administrative staff, peer staff, or billing personnel to make clinical determinations reserved for appropriately qualified professionals.

Procedure

1. The Program Manager coordinates staffing, workflow, productivity, scheduling, vendor/payer operations and program administration but does not independently determine diagnosis, medical necessity, service authorization or clinical disposition unless separately credentialed and authorized.

2. The QP/PSS Program Supervisor directs the Medicaid PSS program, supervises program staff, collaborates on PCP development, monitors beneficiary progress and performs the program-supervisor functions required by policy.
3. The Peer Coordinator may coordinate daily peer operations, schedules, case assignments, documentation follow-up and staff communication. The Peer Coordinator does not replace the QP or assume QP-only duties unless independently qualified and formally designated.
4. CPSS staff provide direct peer interventions within certification scope and documented PCP goals. They shall not diagnose, provide psychotherapy, independently change a treatment plan, or represent themselves as licensed clinicians.
5. The Intake Specialist collects referral, demographic, eligibility and intake information but shall not determine clinical eligibility independently.
6. The Claims Specialist submits claims only after required documentation, eligibility, authorization and coding checks are completed; the Claims Specialist shall not alter clinical documentation to obtain payment.

Required Records: Current job descriptions, organizational chart, delegation records, supervision assignments and access-control matrix.

PSS-003 Staff Qualifications, Credentialing and Ratios

PURPOSE

Ensure every staff member is qualified, appropriately credentialed, and assigned within permitted staffing ratios.

POLICY

ACE Transitions shall credential and verify staff before independent assignment and shall maintain staffing capacity sufficient for beneficiary need, required supervision and service availability.

Procedure

1. The Medicaid PSS program shall be under the direction of a full-time QP meeting applicable 10A NCAC 27G .0104 requirements. A part-time QP shall not serve as the sole arrangement satisfying this requirement.
2. ACE shall maintain a maximum QP-to-CPSS ratio of 1:8, CPSS-to-beneficiary ratio of 1:15, and CPSS group facilitator-to-beneficiary ratio of 1:12.
3. Every individual delivering billable PSS must maintain active North Carolina CPSS certification and must be matched to beneficiaries with similar lived experience as required by policy.
4. Credentialing shall include identity, education/experience where applicable, CPSS certification, exclusion screening, background/reference review as required, payer enrollment or roster status when applicable, and verification of any professional license used for assigned functions.
5. ACE shall maintain an NC CPSS roster showing full name, hire date, initial certification date, recertification date when applicable, and separation date when applicable.
6. A CPSS with expired or revoked certification shall be immediately removed from billable PSS duties until eligibility is restored and verified.
7. Caseloads may be set below maximum ratios based on acuity, travel, documentation burden, schedule, service intensity and staff experience.

Control Standards

- Credential files are reviewed before hire/assignment and at least monthly for time-sensitive items.
- The Program Manager and QP review census and staffing ratios at least monthly and whenever census materially changes.

Required Records: Credential checklist, CPSS roster, verification screenshots/records, exclusion checks, licenses/certifications and staffing-ratio tracker.

PSS-004 Training and Competency Management

PURPOSE

Ensure staff maintain the knowledge, ethics and competencies required for effective PSS.

POLICY

ACE Transitions shall assign and document required onboarding, supervisory and annual training within applicable timeframes.

Procedure

1. Within 30 calendar days of hire to provide service, required staff shall complete review of PSS policy components, documentation training, and opioid-antagonist administration training. The PSS Program Supervisor shall also complete NC Peer Support Supervisor Training within the required 30-day timeframe.
2. Within 60 calendar days of hire to provide service, the PSS Program Supervisor shall complete Person-Centered Thinking and PCP Instructional Elements with Comprehensive Prevention and Intervention Crisis Plan training.
3. All PSS program staff shall complete annual continuing education related to the population served.
4. Prior training may be accepted only when allowed by current policy and supported by documentation showing completion within the permitted look-back period.
5. ACE shall provide additional population-appropriate training such as trauma-informed care, WRAP, WHAM, mental health/substance use fundamentals, Mental Health First Aid, housing/reentry, cultural responsiveness, ethics, boundaries, suicide prevention, documentation and fraud/waste/abuse prevention as operationally appropriate.
6. The QP shall assess performance and competency during supervision and may require remedial training, observation, co-service, documentation correction, or temporary restriction from independent assignments.

Required Records: Training matrix, certificates, competency checklists, supervision notes, remedial plans and annual training report.

PSS-005 Referral, Intake and Eligibility Verification

PURPOSE

Establish a consistent front-door process that protects beneficiary choice and prevents premature service delivery.

POLICY

ACE Transitions shall complete administrative and clinical eligibility checks before assigning billable PSS. Medicaid eligibility must be verified each time a service is rendered.

Procedure

1. The Intake Specialist logs each referral and records referral source, beneficiary contact information, payer, Medicaid ID when available, presenting request and known service providers.
2. Intake shall verify age and payer requirements. Under current Medicaid PSS policy, H0038 PSS is for eligible beneficiaries age 18 and older.
3. Intake shall identify whether another PSS provider is serving the beneficiary during the same episode of care and shall flag potential duplicative or excluded services for QP review.
4. Eligibility verification shall be completed through the applicable payer/NCTracks process before service and rechecked for each date of service through the organization's approved workflow.
5. The QP or qualified clinical staff shall confirm that the beneficiary meets applicable diagnostic and medical-necessity criteria based on the CCA and related clinical documentation.
6. Beneficiary choice of provider shall be preserved. Intake staff shall not guarantee coverage, authorization, payment, placement or outcomes.
7. Incomplete referrals shall remain pending until required documentation or clinical prerequisites are obtained.

Required Records: Referral log, eligibility verification, intake checklist, payer information, releases/consents and referral disposition.

PSS-006 Admission, CCA, Service Order and Person-Centered Plan

PURPOSE

Ensure PSS begins only after required clinical foundations are established.

POLICY

ACE Transitions shall not render or bill Medicaid PSS without a current CCA supporting medical necessity, a valid service order, and a recovery-oriented PCP containing PSS goals and interventions.

Procedure

1. A licensed clinician shall complete the CCA before PSS begins. An equivalent current assessment may be used only if it contains required elements and is acceptable under governing requirements.
2. The service order must be signed by an independently licensed/authorized practitioner permitted by policy prior to or on the first date of service. Backdating is prohibited.
3. The PCP shall connect PSS interventions to identified recovery needs, goals, strengths, barriers, preferences, integrated-care considerations, crisis planning and transition/discharge planning.
4. The QP shall collaborate with the beneficiary and CPSS in developing or operationalizing the recovery-oriented PCP within the QP's scope and organizational workflow.
5. Service orders are tracked for expiration. Current policy makes PSS service orders valid for nine consecutive months; medical necessity and service ordering must be reviewed at least every nine months.
6. When services continue beyond nine months, required updated CCA/addendum, new service order and reauthorization documentation shall be obtained before continued billing.

Required Records: CCA, service order, PCP, consent/release forms, crisis/transition plan and clinical review documentation.

PSS-007 Prior Authorization and Utilization Management

PURPOSE

Control utilization and prevent services beyond authorized limits.

POLICY

ACE Transitions shall verify unmanaged-unit availability and obtain payer authorization when required before providing units that require prior approval.

Procedure

1. Current NC Medicaid policy permits 24 unmanaged units once per episode of care; prior approval is required beyond the unmanaged-unit limitation unless a health plan applies a less restrictive rule.
2. Authorization requests shall include the CCA, service order, PCP, required authorization form and other supporting records required by the payer.
3. ACE shall track units by beneficiary, date, authorization span, payer and service type. Staff shall not rely on memory or verbal assumptions about remaining units.
4. Current policy may cover up to 176 units over a 60-calendar-day authorization period, generally not exceeding 88 units per 30-calendar-day period. Requests exceeding 88 units in a 30-day period require the applicable review/reauthorization process.
5. Prior authorization is not a guarantee of payment. Eligibility, coding, provider status, documentation and claim requirements must still be satisfied.
6. When authorization is denied, reduced, suspended or terminated, required notices and appeal-right information shall be handled according to payer requirements and beneficiary rights procedures.

Control Standards

- The Claims Specialist shall place claims on hold when authorization data are missing or inconsistent.
- The QP and Program Manager review utilization variance, high-unit cases and expiring authorizations at least weekly.

Required Records: Authorization tracker, approval/denial notices, utilization reports, supporting submissions and appeal documentation.

PSS-008 Service Delivery Standards and Scope

PURPOSE

Define what constitutes appropriate, recovery-oriented PSS and prevent noncovered or out-of-scope activity.

POLICY

PSS shall be individualized, goal-linked, recovery-oriented, strengths-based, culturally responsive and delivered within CPSS scope and the beneficiary's PCP.

Procedure

1. Permitted service activities may include peer mentoring/coaching, recovery-resource connection, recovery skill building, community integration, self-advocacy, wellness/recovery tools, system navigation, crisis-prevention support and transition/reentry skill development when tied to PCP goals.
2. PSS may be delivered individually or in an authorized group format and should generally be community based rather than office centered.
3. CPSS staff shall not provide psychotherapy, diagnosis, medication management, formal clinical assessment, legal advice, financial management, childcare, companion/supervision-only services, or services outside certification scope.
4. Transportation itself is not a covered PSS component. Staff may accompany a beneficiary when appropriate to a covered peer-support intervention, but billing shall reflect the covered intervention rather than transportation.
5. Text messages, application-based chat and email correspondence are not billable PSS under current policy.
6. PSS shall not duplicate another Medicaid service or be delivered during prohibited overlapping services/timeframes. Staff shall escalate potential duplication to the QP before service.
7. PSS is not a first-responder service. Crisis response must be coordinated with appropriate clinical/crisis resources.

Required Records: PCP, schedules, service notes, coordination records, referral records and nonbillable-contact log when used.

PSS-009 Caseload Assignment, Supervision and QP Monitoring

PURPOSE

Establish safe caseloads, consistent supervision and documented program-supervisor oversight.

POLICY

ACE Transitions shall assign beneficiaries based on lived-experience fit, geography, availability, acuity, conflicts, staff competency and ratio limits. Administrative coordination does not replace required QP supervision.

Procedure

1. The Peer Coordinator may propose assignments; the QP retains authority to approve or modify assignments for clinical/programmatic appropriateness.
2. The QP/PSS Program Supervisor shall provide administrative and supportive individual supervision to program staff at least every other week, and more often when needed based on experience, risk, performance or case complexity.
3. Supervision shall address caseload, PCP alignment, boundaries, ethics, documentation, engagement, safety, service intensity, resource barriers, staff wellness and needed corrective actions.
4. The QP shall complete at least one in-person, telehealth, or telephonic contact with each beneficiary within 60 calendar days of PSS initiation and at least every 60 calendar days thereafter to monitor progress, program effectiveness, PCP goals and transition/discharge planning.
5. The QP shall conduct routine service-note reviews for quality assurance, monitor staff performance and facilitate team meetings.
6. The Peer Coordinator may conduct operational check-ins and workflow coaching between QP supervision sessions, but these contacts do not substitute for required QP supervision.
7. Supervision shall be documented contemporaneously in the approved supervision record.

Required Records: Assignment roster, caseload report, QP contact tracker, supervision notes, team-meeting records and performance/corrective-action records.

PSS-010 Documentation and Service Note Standards

PURPOSE

Ensure every billed service is supported by complete, accurate and timely documentation.

POLICY

The staff member who provided the service is responsible for documenting that service. Records shall never be falsified, backdated or altered to create eligibility for payment.

Procedure

1. A full service note is required for each contact/intervention for each date of service and must be signed by the person who provided the service.
2. Each service note shall include beneficiary name, Medicaid ID, date, service, contact type, place of service, purpose linked to PCP goals, intervention description, duration with start/end time, assessment of effectiveness/progress, and date/signature with credentials or job title.
3. The note must accurately support the duration billed and demonstrate a covered peer intervention rather than merely travel, waiting, companionship, scheduling, administrative activity or general conversation.
4. Corrections shall follow a transparent amendment process preserving the original entry, date, author and reason for correction. Backdating is prohibited.
5. The Peer Coordinator may perform first-level completeness checks. The QP performs clinical/program quality review. Neither reviewer may rewrite a CPSS note to create a billable service.
6. Late or incomplete notes shall be returned for correction and claims shall be held until documentation supports billing.

Control Standards

- No claim is released before documentation, eligibility, authorization and coding checks are complete.
- Repeated documentation deficiencies trigger coaching, retraining or progressive corrective action.

Required Records: Service notes, amendment logs, documentation review logs, correction requests and audit findings.

PSS-011 Telehealth and Telephonic Services

PURPOSE

Use remote modalities only when clinically appropriate, secure and permitted.

POLICY

ACE Transitions may use telehealth or telephonic audio-only PSS when allowed by current policy, payer requirements and beneficiary needs. Remote service is supplemental and shall not replace required community-based engagement.

Procedure

1. Before telephonic/audio-only encounters, staff shall confirm appropriateness, safety, beneficiary capability, consent, privacy and identity using two points of identification.
2. Telehealth shall use approved secure technology and applicable modifiers. Telephonic claims shall use the applicable audio-only modifier required by current policy.
3. Current policy limits combined telehealth and telephonic audio-only time to 10 percent or less of total service time per beneficiary per fiscal year. The Claims Specialist shall monitor this threshold.
4. Group PSS (H0038 HQ) is not telehealth or telephonic eligible under the current 8G claims attachment.
5. Remote encounters shall be documented with modality, place of service, intervention, start/end time and other required service-note elements.
6. Digital text messages, email and application chat are not billable PSS.

Required Records: Telehealth consent, identity verification documentation, modality utilization tracker, service notes and claim modifiers.

PSS-012 Safety, Crisis Response and Incident Escalation

PURPOSE

Protect beneficiaries and staff while preserving the non-clinical scope of peer support.

POLICY

PSS staff shall recognize urgent concerns and immediately follow ACE emergency procedures. PSS is not a first-responder or crisis service.

Procedure

1. CPSS staff shall identify and promptly escalate suspected abuse/neglect, suicidal or homicidal risk, exploitation, medical emergencies, significant intoxication/overdose risk, violence, missing-person concerns or other urgent safety conditions.
2. Immediate danger requires activation of emergency/crisis resources consistent with law, the PCP/crisis plan and organizational procedure. Staff shall not delay emergency response while waiting for routine supervisory approval.
3. The QP and Program Manager shall be notified according to the incident severity matrix. Payer, guardian/LRP, regulator, law enforcement or other reporting shall occur when required.
4. After a mental health or SUD crisis-service discharge, the CPSS and QP should collaborate with the beneficiary to review recovery tools, crisis-prevention strategies and the crisis plan as clinically/operationally appropriate.
5. Incidents shall be documented separately from the service note when an incident-report process applies. Documentation must be factual, objective and timely.
6. Post-incident review shall identify follow-up, staff support, training, policy or quality-improvement actions.

Required Records: Incident reports, crisis referrals, mandated-reporting documentation, follow-up notes, debrief records and corrective actions.

PSS-013 Confidentiality, Privacy and Information Security

PURPOSE

Protect beneficiary information and restrict access to authorized business and care purposes.

POLICY

ACE Transitions shall protect confidential information consistent with HIPAA, 42 CFR Part 2 when applicable, NC requirements, payer contracts and organizational policy.

Procedure

1. Access to records shall follow least-privilege principles and role-based permissions.
2. Staff shall use only approved EHR, communication, cloud, phone and file-storage systems for protected or sensitive information.
3. Releases of information shall be verified for scope, recipient, purpose and expiration before disclosure unless disclosure is otherwise permitted or required by law.
4. Staff shall not place identifiable beneficiary information into unapproved artificial-intelligence, personal email, consumer cloud, messaging or test systems.
5. Privacy incidents, misdirected communications, lost devices, unauthorized access or suspected breaches shall be reported immediately through the incident/security process.
6. Conversations about beneficiaries shall occur only in appropriate private settings and only with individuals who have a legitimate authorized need to know.

Required Records: Privacy acknowledgments, releases, access logs, security incident records, breach-risk assessments and access review records.

PSS-014 Professional Boundaries, Ethics and Conflicts of Interest

PURPOSE

Preserve the integrity of peer relationships and prevent dual relationships, exploitation and conflicts.

POLICY

All CPSS and supervisors shall adhere to the NC CPSS Code of Ethics and Conduct and ACE boundary standards.

Procedure

1. CPSS staff shall not exchange personal loans, money, housing arrangements, unauthorized transportation, gifts of material value, romantic/sexual relationships, business opportunities or personal favors with beneficiaries.
2. Staff shall disclose actual or potential conflicts promptly and request reassignment when objectivity, safety, boundaries or personal well-being may be compromised.
3. CPSS shall not provide PSS to their direct supervisor, manager or anyone who influences their employment status.
4. Whenever possible, CPSS shall not provide PSS to coworkers in the same program/work unit. Any unavoidable employee-service arrangement requires advance leadership approval, a documented conflict assessment, written confidentiality-limit notice and ongoing supervisory monitoring.
5. Ethical concerns involving CPSS certification shall be escalated and reported to the NC CPSS Program when required.
6. Boundary violations may result in reassignment, corrective action, reporting, suspension from service or termination depending on severity.

Required Records: Conflict disclosures, reassignment records, employee-recipient assessments, ethics reports and corrective-action documentation.

PSS-015 Billing, Claims and Revenue Integrity

PURPOSE

Ensure claims reflect services actually delivered, documented, authorized and billable.

POLICY

ACE Transitions shall bill only for covered PSS delivered by an active NC CPSS and supported by the beneficiary record. QP/program-supervisor services and supervision are indirect costs and shall not be billed separately as H0038.

Procedure

1. The Claims Specialist shall verify beneficiary eligibility, provider/payer status, authorization, date of service, service code, units, modifier, place of service and documentation before release.
2. H0038 is billed in 15-minute units. H0038 HQ is the group code and must be billed according to current payer and NC Medicaid requirements.
3. Claims shall not include noncovered activities such as transportation-only time, uncovered text/email contacts, services not rendered, supervision, administrative work, duplicate services or interventions not tied to PCP goals.
4. Denials shall be categorized, corrected when appropriate, appealed when supported, and trended for root-cause analysis.
5. Overpayments, duplicate payments, unsupported claims or suspected fraud/waste/abuse shall be escalated immediately and handled under applicable repayment/self-report requirements.
6. Clinical staff may clarify or correct their own documentation through the approved amendment process; billing staff shall never direct staff to fabricate, backdate or materially alter documentation to secure payment.
7. Internal audits of units and records shall be performed routinely and results reported to leadership.

Required Records: Claim files, remittances, denial logs, authorization records, audit samples, refund/overpayment records and corrective actions.

PSS-016 Participant Rights, Complaints and Grievances

PURPOSE

Protect beneficiary dignity, choice, access to information and ability to raise concerns without retaliation.

POLICY

ACE Transitions shall provide accessible processes for complaints and grievances and shall preserve payer appeal rights when services are denied, reduced, suspended or terminated.

Procedure

1. At intake, beneficiaries shall receive information on rights, confidentiality, communication expectations, complaint/grievance channels, emergency resources and how to contact their health plan when applicable.
2. Complaints may be verbal or written and shall be logged, acknowledged and routed according to severity and subject matter.
3. Clinical or safety complaints are escalated to the QP; billing/financial concerns to the Program Manager/Claims Specialist; privacy concerns to the privacy/compliance lead; employee-conduct matters to leadership.
4. No beneficiary shall be retaliated against for making a complaint, grievance, appeal or good-faith report.
5. Resolutions shall be documented and communicated within the applicable organizational or payer timeframe.
6. When a payer action triggers formal appeal rights, staff shall not substitute the internal grievance process for the payer's required notice and appeal process.

Required Records: Rights acknowledgment, complaint/grievance log, investigation notes, resolution letters and payer appeal records.

PSS-017 Transition, Discharge and Continuity of Care

PURPOSE

Plan for safe transition from the beginning of service and support continuity of recovery.

POLICY

Transition and discharge planning begins at admission and shall be documented throughout the episode of care.

Procedure

1. The PCP/service record shall contain a transition/discharge plan developed with the beneficiary.
2. The QP/PSS Program Supervisor shall review transition/discharge planning with the beneficiary at least every 60 calendar days in connection with required progress monitoring.
3. Discharge may occur when goals/functioning have sufficiently improved, recovery supports are stable and PSS is no longer needed, a more intensive service is indicated, or the beneficiary/LRP chooses to withdraw.
4. When routine mental health needs are identified, ACE shall link the beneficiary to needed mental health services within 14 calendar days. Routine substance-use service needs shall be linked within 72 hours. Urgent/emergent needs require immediate referral to appropriate clinical/crisis resources.
5. Warm handoffs, releases and coordination shall be used when authorized to reduce gaps in care.
6. The final documentation shall summarize progress, remaining needs, referrals, natural/community supports and follow-up recommendations within the CPSS/QP scope.

Required Records: Transition/discharge plan, referral confirmations, final service note, discharge summary when used and closure checklist.

PSS-018 Quality Assurance, Auditing and Corrective Action

PURPOSE

Use data and record review to identify risk, improve service quality and protect billing integrity.

POLICY

ACE Transitions shall maintain an ongoing quality-management process that reviews documentation, service delivery, utilization, staffing, credentials, beneficiary outcomes, incidents, complaints and claims.

Procedure

1. The QP shall perform routine service-note and caseload reviews. The Program Manager shall maintain operational dashboards for census, staffing ratios, timeliness, authorizations, documentation completion, claims status and training.
2. At least monthly, leadership shall review a sample of beneficiary records for CCA/service order/PCP presence, authorization, service-note completeness, unit support, QP monitoring, transition planning and billing consistency.
3. High-risk findings - including unsupported billing, expired credentials, missing service orders, ratio violations, repeated late notes, privacy breaches or unsafe practice - require immediate containment and corrective action.
4. Corrective-action plans shall identify the finding, responsible owner, required action, due date, validation method and closure date.
5. Trends shall be reported to executive leadership and, when material, the Board or appropriate committee.
6. Staff may be removed from assignments or billing while serious findings are investigated.

Required Records: QA dashboard, audit tools, findings log, CAPs, training records, validation reviews and board/leadership reports.

PSS-019 Clinical Integration and External Provider Agreements

PURPOSE

Ensure PSS is integrated with access to appropriate clinical services.

POLICY

ACE Transitions shall maintain the clinical integration required by current NC Medicaid policy and shall not operate PSS as a stand-alone substitute for needed clinical care.

Procedure

1. The provider organization shall offer, at minimum, access to comprehensive clinical assessment, psychotherapy and medication management as required by current policy.
2. If medication management is not provided within the organization, ACE shall maintain a signed MOU/MOA with an appropriate medication-management provider before providing PSS when required for newly enrolling providers, and shall submit the agreement to contracted health plan(s) as required.
3. Beneficiaries retain freedom of provider choice and are not required to obtain clinical services from ACE or an MOU/MOA partner.
4. Staff shall collaborate with care managers, clinical homes, health plans and other service providers when authorized and appropriate.
5. Referrals shall be tracked through completion or documented barrier/declination.

Required Records: MOU/MOA, payer submission confirmation, referral log, coordination notes and provider directory.

PSS-020 Records, Policy Review and Change Control

PURPOSE

Maintain reliable records and controlled policy governance.

POLICY

ACE Transitions shall maintain records in accordance with applicable federal/state law, Medicaid/payer requirements, accreditation standards and organizational retention policy. This manual shall be a controlled document.

Procedure

1. The Program Manager shall maintain the master policy copy, version number, effective date, approval record and revision history.
2. Obsolete versions shall be archived and removed from active staff use while retained according to organizational record-retention requirements.
3. Staff shall receive notice and training when changes affect their duties, documentation, billing, safety or beneficiary rights.
4. Electronic records shall be backed up and protected by role-based access, multifactor authentication and approved security controls.
5. Record destruction shall be suspended when litigation hold, audit, investigation, repayment, credentialing or other legal preservation requirement applies.
6. Questions about interpretation shall be escalated to the QP, Program Manager, Executive Director, payer/provider-relations representative or legal/compliance counsel as appropriate.

Required Records: Policy index, version history, training acknowledgments, access logs, retention schedule and legal-hold records.

Appendix A - Operational Checklists

Pre-Service Admission Checklist

- ☐ Referral entered in intake log
- ☐ Age and identity verified
- ☐ Current Medicaid/payer eligibility verified
- ☐ No conflicting PSS provider/duplicative-service concern identified
- ☐ CCA completed by qualified licensed clinician
- ☐ PSS medical necessity supported
- ☐ Valid service order signed on/before first service date
- ☐ PCP includes PSS goals/interventions
- ☐ Required consent/release documents completed
- ☐ Authorization/unmanaged units verified
- ☐ CPSS certification active
- ☐ Lived-experience fit and assignment approved
- ☐ Crisis/transition information available to assigned staff

Pre-Claim Checklist

- ☐ Service provided by active NC CPSS
- ☐ Eligibility verified for date of service
- ☐ Authorization/unmanaged units available
- ☐ Service note signed by rendering CPSS
- ☐ Start/end time supports units billed
- ☐ PCP goal linkage documented
- ☐ Intervention and beneficiary response/progress documented
- ☐ Place of service and modality documented
- ☐ Correct code/modifier selected
- ☐ No duplicate/overlapping prohibited service
- ☐ Telehealth/telephone threshold checked when applicable
- ☐ Claim released only after all edits clear

Monthly QA Checklist

- ☐ QP-to-CPSS and CPSS-to-beneficiary ratios reviewed
- ☐ CPSS roster/certifications verified
- ☐ Expiring service orders and authorizations reviewed
- ☐ QP 60-day beneficiary contacts tracked
- ☐ Biweekly supervision completion reviewed
- ☐ Training deadlines reviewed
- ☐ Late/incomplete notes trended
- ☐ Claims denials and overpayments reviewed

- ☐ Incidents/grievances/privacy events reviewed
- ☐ Referral linkage timeliness reviewed
- ☐ MOU/MOA and payer requirements current
- ☐ Corrective actions monitored to closure

Appendix B - Required Forms and Logs

- Referral and Intake Form
- Eligibility Verification Record
- Authorization / Unit Tracking Log
- CPSS Credential Verification Checklist and CPSS Roster
- Staff Training Matrix
- Beneficiary Assignment / Caseload Roster
- Individual Supervision Record
- QP 60-Day Beneficiary Monitoring Log
- Service Note Template
- Documentation Correction / Amendment Record
- Incident Report
- Complaint and Grievance Log
- Conflict-of-Interest / Employee-Recipient Assessment
- Transition and Discharge Checklist
- Claims Edit / Pre-Bill Checklist
- Denial and Appeal Log
- Monthly Quality Audit Tool
- Corrective Action Plan
- External Referral and Linkage Tracker
- MOU/MOA Register
- Policy Revision Log

Appendix C - Authority and References

Authority	Relevance	Source
NC Medicaid Clinical Coverage Policy 8G - Peer Support Services	Amended September 1, 2026	https://medicaid.ncdhhs.gov/8g-peer-support-services
NC Medicaid Program-Specific Clinical Coverage Policies	Current provider policy index	https://medicaid.ncdhhs.gov/providers/program-specific-clinical-coverage-policies
NCTracks Provider Manuals and Billing Guidance	Current claims and enrollment guidance	https://www.nctracks.nc.gov/content/public/providers/provider-manuals.html
42 CFR Part 2	Confidentiality of substance use disorder patient records, when applicable	https://www.ecfr.gov/current/title-42/chapter-I/subchapter-A/part-2
HIPAA Privacy and Security Rules	Federal privacy/security requirements	https://www.hhs.gov/hipaa/index.html